

Direct Primary Care

A Physician-Owned Growth Model

Small practices, protected clinical time and shared nonclinical support

For discussion with physicians, operators, advisors and potential investors

The concept needs proof at the practice and support-company levels

Practice proposition

Patients pay for continuity and access. Physicians receive fair compensation, manageable work and a potential path to ownership.

Growth proposition

Shared services must deliver more value than they cost. Repeatable openings and retained practices would support expansion.

Current status: an operating hypothesis and working model. Roles, partners, ownership and funding remain undecided.

The proposed experience requires a premium-value test

Established DPC context	This proposal to test
Periodic membership for primary care	Core \$200 / Extended \$300 per month
Care outside traditional insurance billing	Protected time and planned relationship visits
Physicians may be employees or owners	One owner and up to two associates
Practice structures and services vary	A mentored ownership path and shared support

AAFP describes a general \$50-\$100 monthly range. The proposed \$200-\$300 needs evidence that its experience is worth the premium.

Sources: AAFP DPC overview and 2024 data brief context; DPC Frontier definition. Reference prices are hypotheses, not local market quotes.

Continuity and dependable access must justify the membership

Offer to test

Core: 25 physician minutes in a 30-minute block. Extended: 45 minutes in a 60-minute block. A monthly check-in is optional and guided by need.

Evidence to collect

Interview likely buyers about alternatives and budget. Test paid demand and reasons for staying. Track useful care plans, access and the work between visits.

Extra-visit charges and included services remain design questions. Membership does not replace coverage for hospital or specialist care.

Protected time limits the panel that the schedule can support

Mature reference schedule	Assumption
Owner / each associate	3 / 5 days per week, 46 weeks per year
Working day	9 hours, including 1 hour each for break, urgent care and admin
Routine appointments	6 per physician day, subject to available hours
Monthly routine demand / capacity	252 / 299 visits at 360 members and 70% attendance

The 85% access target allows 363 members under this mix. This arithmetic does not establish a clinically safe panel or complete after-hours coverage.

Illustrative assumptions for expert review. No practice affiliation, operating data or commitments implied.

A credible ownership path needs clear readiness and financing terms

Proposed progression

Join a physician-led practice. Develop relationships and operating skills. Qualify through care quality and business readiness. Explore ownership of a new or existing location.

Unresolved terms

Buy-in valuation, funding, transition coverage, governance, exit rights and succession. The 18-36 month window is a planning hypothesis, not a promised promotion.

Drew developed the concept. Future founder roles, ownership percentages and compensation remain open.

The reference practice earns \$78,000 after physician pay and platform fees

Annual economics at 360 members	USD
Membership revenue at \$275 blended monthly dues	\$1,188,000
Physician cash pay and employment costs	(\$840,000)
Local nonphysician operating costs	(\$210,000)
Support-company fee	(\$60,000)
Operating surplus / margin	\$78,000 / 6.6%

Before depreciation, financing costs and tax. Startup spending and the support company’s own costs remain separate.

Illustrative assumptions for expert review. No practice affiliation, operating data or commitments implied.

The margin target requires more members than the reference access target allows

Enrollment threshold	Members
Break-even at the reference price and costs	337
Maximum within the 85% utilization target	363
Required for a 15% operating margin	396

The gap requires a better price, cost or capacity combination. Filling more appointments alone would compromise the selected access target.

The opening plan adds physicians as enrollment grows

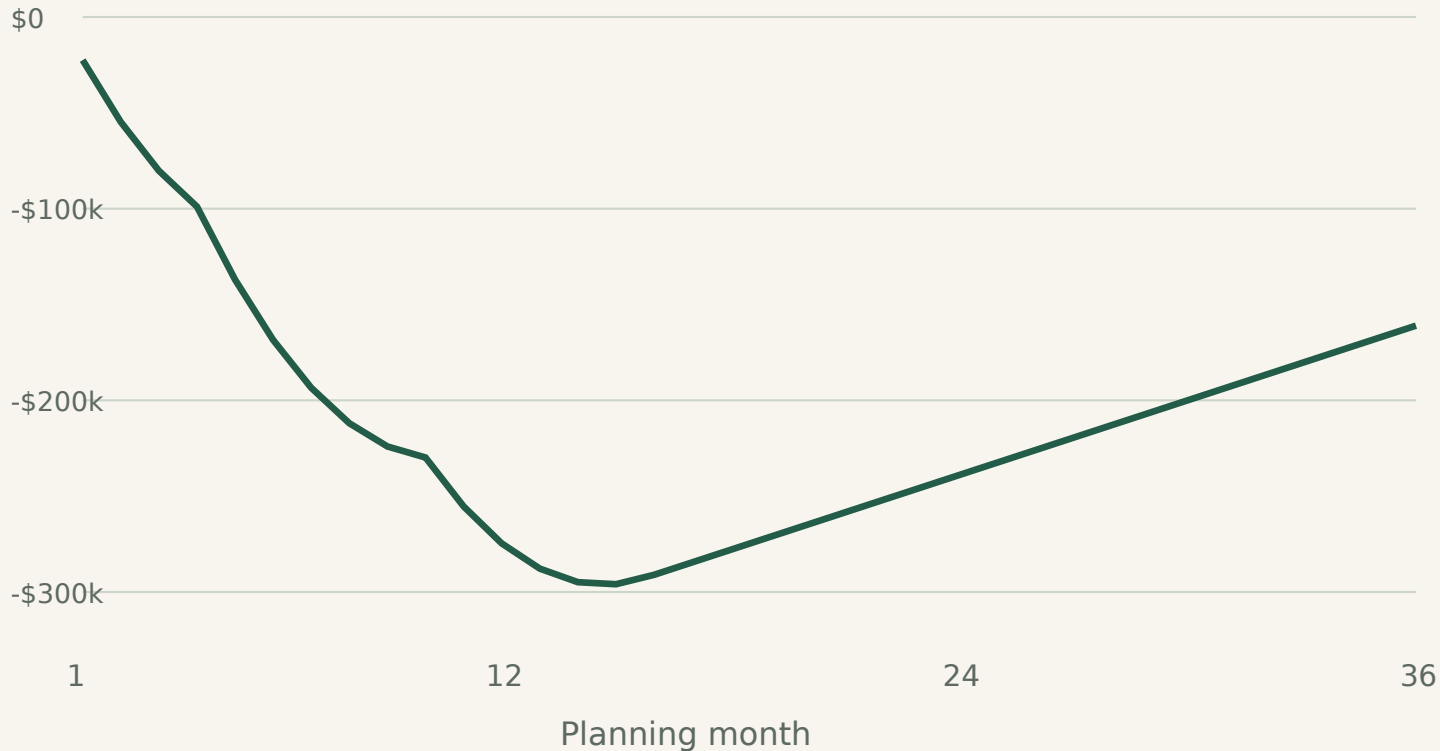
Timing input	Illustrative reference
Planning horizon / care begins	36 months / month 2
Initial / subsequent gross enrollments	30 at opening / 25 per following month
Monthly attrition / mature enrollment target	1% / 360 members
Owner / first associate / second associate	Months 2 / 5 / 11
Local costs and platform fee	Full monthly run rate from month 1

Hiring dates are independent inputs. The tool flags any period when enrollment outruns routine capacity.

Illustrative assumptions for expert review. No practice affiliation, operating data or commitments implied.

Monthly break-even arrives before the opening losses are recovered

Cumulative operating cash before startup spending



Month 16

First operating break-even that holds through month 36

-\$295,941

Peak operating cash deficit in month 15. Still about \$161,000 below zero at month 36.

The total opening funding requirement remains incomplete

Known in the illustration

The enrollment ramp and recurring costs create an operating funding requirement near \$296,000 before setup costs, reserves or financing. This is a lower bound for this scenario.

Still required

Equipment, deposits and setup quotes. Opening cash. A target reserve. Financing terms and repayment timing. These inputs remain blank or explicitly zero until supplied.

Startup spending occurs once. Opening cash and financing are sources. A reserve is a target balance, not another expense.

Annual payment changes cash timing and can sharply reduce earned margin

Mature reference case	Annual surplus	Margin
All monthly payments	\$78,000	6.6%
25% prepay with a 20% discount	\$18,600	1.6%
All prepay with a 20% discount	-\$159,600	-16.8%

The launch model collects annual dues on joining and renewal, earns them monthly, and refunds unused months after cancellation. Unearned dues remain an obligation.

Practices must receive measurable value for the proposed service fee

Proposed services

Administrative workflows, scheduling systems, vendor coordination, recruiting support, operating reports and practice-owner development. Service scope and delivery costs require quotes.

Practice value test

Measure hours saved, service reliability and opening effort. Compare the fee with a realistic independent alternative. Practices retain clinical judgment, care quality and patient relationships.

Reference fee: \$5,000 per location per month. No assumed clinical control, finalized contract or legal safe harbor.

Practice sales and support-company revenue belong to different businesses

Equally mature locations	Practice revenue / year	Platform revenue / year
1	\$1.188 million	\$60,000
25	\$29.7 million	\$1.5 million
100	\$118.8 million	\$6.0 million

Platform surplus = service fees minus delivery costs and central overhead. Both cost inputs remain unknown. Combined surplus eliminates the fee transfer.

An investable support company needs proven earnings and enforceable boundaries

Possible sources of return

Physician owners could earn practice distributions after obligations.
Support-company investors could earn returns from its independently validated profits and value. No investor return or exit multiple is assumed.

Decisions before financing

Who founds and owns each business?
What does the fee buy? Who funds new locations? What clinical boundaries and exit rights apply? What capital type fits the cash profile?

Evidence should determine the next commitment

Hypothesis	Evidence and reviewer	Decision informed
Patients value the offering	Paid demand and retention; patients and DPC operator	Price and included care
The work fits the schedule	Time study incl. messages; physician lead	Panel, staffing and access
Physicians want the pathway	Recruiting interviews and terms; physician operators	Compensation and ownership
Support justifies its fee	Costed service trial; practice operator	Fee and platform scope
Launch cash and structure work	Quotes and scenarios; finance and healthcare counsel	Funded pilot, then expansion

A pilot can narrow the uncertainty before network expansion

Proposed gates to test

Observe 25 opt-in members for 90 days each. Test 90% retention, agreed care plans and routine replies within the selected coverage target. Keep routine utilization at or below 85%.

Conditions still unresolved

Define the actual offering and response promise. Fund the launch. Validate physician terms. Meet a sustainable margin and access target together, then observe stable performance before another site.

Tax treatment, membership terms, clinical ownership and possible franchise obligations require current specialist review. The concept makes no eligibility or outcome promises.

The next version should reflect expert criticism and better inputs

AAFP: category, career options and general pricing context

DPC Frontier: category definition

IRS Notice 2026-5: distinct HSA contribution and reimbursement rules

Texas Medical Association: clinical independence background

FTC: franchise-rule guidance

Editable comparison, assumptions, notes and export

Prepared by Drew Cleaver. Founder roles, ownership, compensation and partnerships remain undecided.